
PREPARED FOR CARBON

Hospital receivables, verified, recovered and funded

*A partnership in which Carbon lends against claims the health plans owe,
and we make sure they pay.*

PREPARED FOR

Chijioke Dozie, Carbon

PREPARED BY

Dr Debo Odulana

MEETING

8 October 2026



Carbon lends against claims the plans owe. We make sure the plans pay.

60%

of a verified claim's value paid to the hospital within 48 hours

8

checks every claim passes before any money moves, and a person signs each one

799

Lagos providers already in our directory, with a direct line to 702 of them

Live

public page, claims desk and decision rules are running in production today

WHAT CARBON DOES

- Lends to the hospital against its verified claims, through a collection account the plans pay into.
- Sets the credit policy, the facility per hospital and the discount.
- Receives claim-level performance data, monthly or live.

WHAT WE DO

- Find the hospitals, verify every claim and keep the bad ones out of the book.
- Settle disputes with the plans, chase, escalate and collect.
- Never lend, never buy a claim, never guarantee. Factoring needs a licence we do not hold and do not want.

For tomorrow: agree the lending route, the pilot size, how the discount is set and what data Carbon needs to underwrite.



Private hospitals are financing the health plans.

Most of what a private hospital earns from insured patients is owed by HMOs, with corporate accounts and state schemes behind them. The work is done and the cost is paid this month. The cash arrives months later, if the claim survives the plan's review at all.

56 days

average time to process a claim on the largest Nigerian HMO claims platform in 2025, down from 81 days in 2024

8.4m

claims a year through that one platform, worth about N22bn, across 20 to 25 HMOs

30 days

the most the Guild of Medical Directors said in 2022 its members would tolerate being owed, with interest after

"They have not paid us in three months after treating more than 250 patients."

A Lagos private facility manager, in a study of why facilities hesitate to join insurance schemes

Sources: Medismarts figures as reported by Streamlinefeed, July 2026 (vendor-reported; the article does not say whether this is submission to payment or to adjudication). Guild of Medical Directors, BusinessDay and Daily Trust, January 2022. Facility manager: PLOS ONE, 2021, fieldwork in Lagos 2017 to 2018. We found no published Nigerian rejection rate. The pilot will produce the first one.



Most of it is disputed. Very little of it is refused.

Why the claim is unpaid	What clears it	Recoverable
No pre-authorisation code, or the code does not match the service billed	Retrospective authorisation, agreed with the plan's medical team	Usually
Billed above the agreed tariff, or on an old tariff	Rebilled at the agreed rate; the gap is written off, not the claim	Mostly
Missing documents: notes, results, discharge summary	Pull the record and resubmit inside the plan's window	Usually
Submitted late, or never submitted	Depends on the plan's window and goodwill	Sometimes
The plan is slow to pay across the board	Escalation, a payment schedule, then the regulator	Slowly
The enrollee was not covered, or the plan has failed	Bill the patient or employer, or write it off	Rarely

This is why the risk is better than it looks from outside. A claim that has been verified is a debt from a regulated payer for work already done, and most of what blocks payment is paperwork that can be fixed before Carbon lends a naira.



The law, the rate cycle and the failures all point the same way.

Late payment is an offence

The NHIA Act 2022 makes it an offence for an HMO to fail to pay providers or settle claims within the period in the operational guidelines (s.48), and sends disputes to NHIA mediation first (s.47). Recovery has a regulator behind it.

Receivables are assignable

The Secured Transactions in Movable Assets Act 2017 makes an assignment of a receivable effective against the debtor even where the contract limits assignment (s.4(2)), with priority by registration at the National Collateral Registry (s.33).

A Factoring Act is close

The Factoring, Assignments and Receivables Financing Bill 2026 passed the National Assembly in June 2026 and went for assent. We could not confirm it has been signed.

Unsecured SME credit has failed

Lidya closed in October 2025 on loan losses and naira depreciation. Lending repaid from a named, regulated payer's cash is a different risk, which is the case for this book.

Sources: NHIA Act 2022 (Gazette No. 95, May 2022); STMA 2017; Senate proceedings and Afreximbank on the Factoring Bill, June 2026; TechNext, October 2025. The CBN also cut the policy rate from 26.5% to 23% on 22 September 2026, though lenders warn the cost of credit will lag.



Verify, reconcile, recover

One desk, run by people who have run hospitals and sat across the table from health plans, on a platform we built for the job.

Verify

Every claim is read against the plan's tariff, the authorisation record, the documents held and its age. The platform drafts the verdict, the likelihood of payment and the exact repairs; a person confirms it. Claims that will not pay are kept out of the funded book and sent to repair.

Reconcile

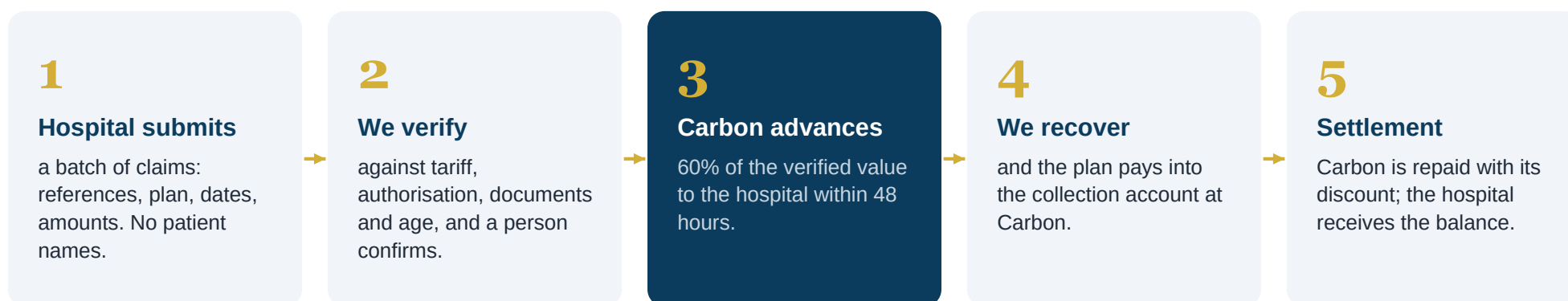
We match what the hospital billed against what each plan says it owes, then clear the gap claim by claim with the plan's claims team. For each plan the platform ranks the claims by naira recovered per hour of effort and drafts the reconciliation letter and the call script.

Recover

We chase, escalate and settle. Every call is logged and the platform turns the notes into dated promises, disputes and next actions, so a promise made on a Tuesday is chased on the Friday. Payment lands in the account Carbon controls.



Sixty per cent within 48 hours, the balance when the plan pays.



ONE CLAIM, END TO END (ILLUSTRATIVE)

	Naira	When
Billed to the plan	1,000,000	Day 0
Verified payable (after tariff correction)	950,000	Day 1
Advance to the hospital, 60% of verified	570,000	By day 3
Plan pays into the collection account	950,000	Day 30 to 90
Carbon: advance returned, plus its discount	570,000 + discount	On receipt
Hospital: the balance, less the recovery fee	the rest	On receipt



Every advance passes the same eight checks, in this order.

#	Check	If it fails
1	Funding is live with a signed funder, and the hospital's agreement is signed and active	Hold
2	The claim has been verified, and a person has confirmed the verification	Hold
3	Verdict is PASS. A claim that needs repair is repaired first; one that will not pay is declined	Hold or decline
4	Likelihood the plan pays is at least 75%	Hold
5	The payer is eligible, and is not one CFA advises	Decline
6	Claim age is within the limit (180 days by default, set per hospital)	Decline
7	The advance keeps the hospital inside its facility limit	Hold
8	No single plan above 40% of the hospital's advanced book, once the book is past a quarter of its limit	Hold

The amount

60% of the lowest of what was billed, the agreed tariff and the verified payable. Rounded down to the kobo.

Who decides

The rules decide. The model's view can stop an advance but never authorise one. A person approves every advance, and Carbon's own credit decision comes after that.



What the desk sees before anyone lends.

Two test claims run through the platform this week. The output below is what it returned, lightly shortened.

Caesarean section, 3 nights

Payable after rebill

Billed N450,000 Tariff N420,000 Authorised 117 days

FOUND

- Authorisation, documents and timing are in order
- Only query: N30,000 billed above the agreed tariff

REPAIRS, IN ORDER

- Confirm N420,000 is the current tariff
- Reissue at N420,000 under the same authorisation code
- Include the operation note with the clinical notes
- Reply to the tariff query in writing and ask for a payment date

Expected payable N420,000

Appendicectomy

FAIL, 12%

Billed N600,000 No tariff on file No authorisation 232 days

FOUND

- Never submitted, 232 days after service
- No authorisation for a procedure that normally needs one
- No documents held, cover not confirmed

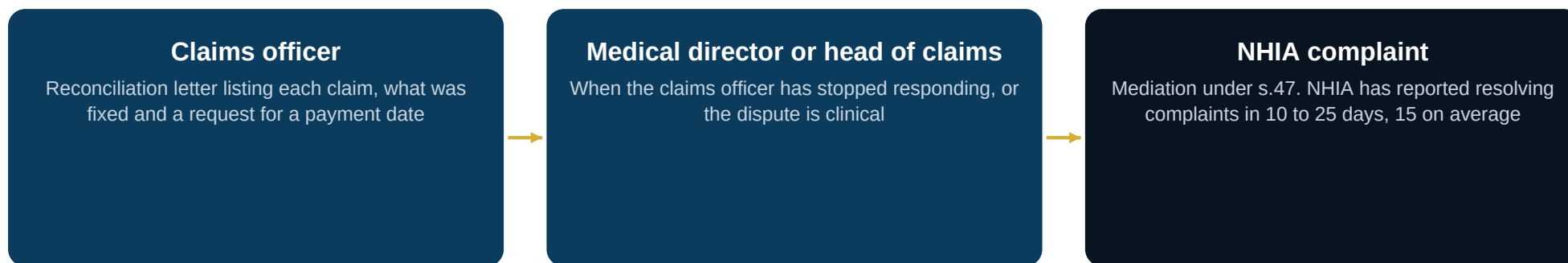
REPAIRS, IN ORDER

- Establish whether it was an emergency and whether the plan was told
- Check the submission window and tariff in the provider agreement
- Only then decide whether to pursue retrospective authorisation

Kept out of the funded book



Reconciliation first, the regulator last.



WHAT THE PLATFORM DOES ON EVERY CALL

- Reads the call notes and records each promise with its amount, its claims and its date
- Logs each dispute against the claim it concerns
- Sets the next action and its due date, so nothing promised is left unchased

FROM A TEST CALL THIS WEEK

"She accepts CL-001 at the tariff of 420k, in the next payment run, end of October. CL-002 has no PA code, so they will not pay unless the medical director approves a retrospective authorisation."

Recorded as: a promise of N420,000 on CL-001 by 31 October; a dispute on CL-002; six follow ups, the first due tomorrow.



Running in production since 7 October

CHECK YOUR RECEIVABLES

What is sitting outside your hospital?

YOUR NUMBERS

Billed to HMOs and corporates each month
N15.0m

Average days until a claim is paid
120 days

Share of claims queried, cut or rejected
20%

N60.0m
of that, owed to you at any one time

N12.0m
of that, held up in queries and disputes

N15.0m
released for good if you are paid 30 days sooner

This is arithmetic on the figures you enter, not a forecast of what we will recover. The written read is.

GET THE WRITTEN READ

Free. We review 20 of your unpaid claims and tell you what is recoverable, from whom, and how fast.

Hospital or clinic

Your name
Your role

Email
Phone (WhatsApp)

City
How many HMOs pay you?

Oldest unpaid claim

Anything we should know (optional)

☐ I am interested in early payment against vetted claims when it opens
☐ Consult for Africa may hold these details to reply to me about this service. We never ask for patient names or records at this stage.

Send me the written read →

Public page

A hospital enters its billing, days to payment and query rate, sees what is sitting outside the business, and asks for a free review of 20 claims.

Directory and funnel

Every hospital, every contact and every stage from target to signed, with call and WhatsApp logging.

The desk

Claims received from a spreadsheet, verified, decided, approved, advanced, chased and settled, with an append-only record of every step.

Data for Carbon

Claim-level status, promises and payments, ready to share as a feed or a monthly file.

consultforafrica.com/services/claims-recovery, live since 7 October 2026: the receivables check and the free review offer

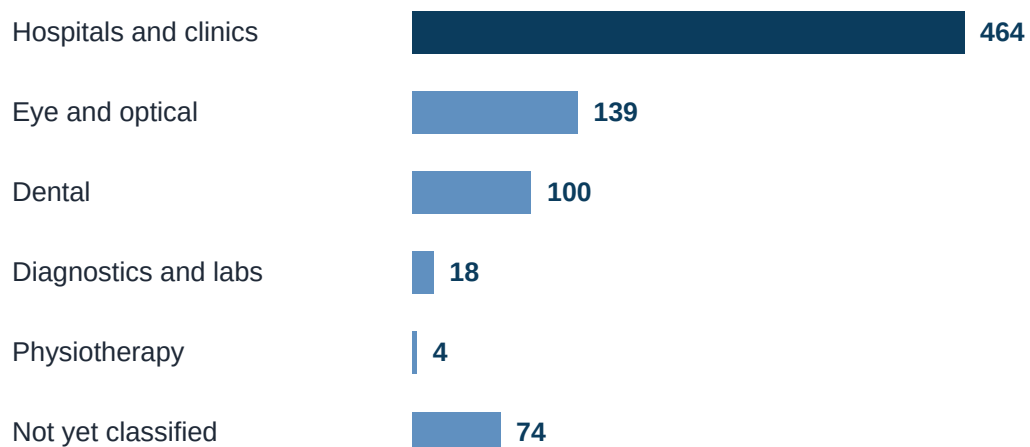
Consult for Africa / Private and confidential / Prepared for Carbon

11 / 20



Where the first book comes from

799 LAGOS PROVIDERS IN OUR DIRECTORY, BY TYPE

**702**

with a working email on file

698

with a direct phone number

21

local government areas covered

190

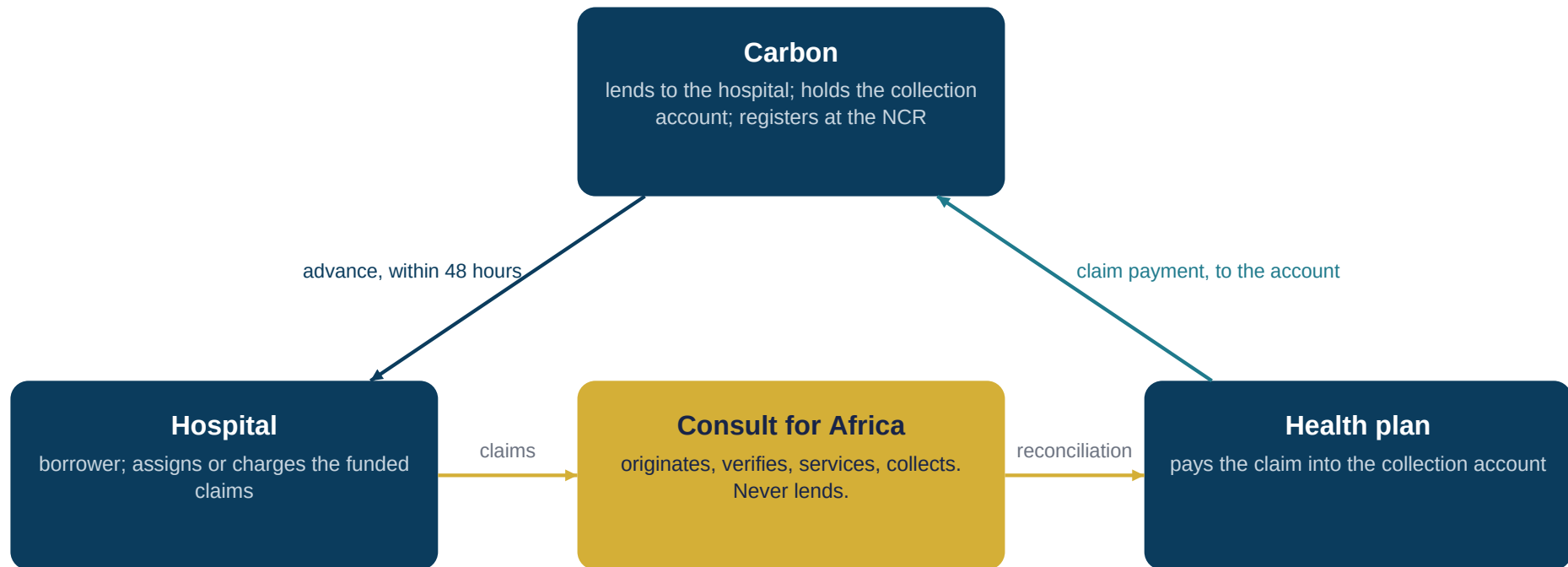
in Eti-Osa alone, the densest private market

HOW WE REACH THEM

- Direct: a call and a WhatsApp from a named person, then the free review of 20 claims. The review is the conversion step, and it costs the hospital nothing.
- Through the profession: the Guild of Medical Directors and the private practitioners' associations have campaigned on HMO debts since at least 2022. One endorsed member offer is worth more than the whole cold list.
- Through our own work: hospitals whose operations and accounts we already run or have audited.



How Carbon gets to the cash



- Collection account at Carbon with a domiciliation and lien; the plan is told to pay there.
- Register every funded claim at the National Collateral Registry. Priority goes by time of registration (STMA s.33(3)).
- Serve notice of assignment on a plan only on default, slow payment or a set-off risk, so Carbon stays out of the hospital's relationship with the plan.
- Vet for open clawbacks: set-off the plan acquires before notice binds the lender (s.33(1)(b)).

For Carbon's counsel to confirm: whether Carbon lends against the claims or buys them outright, and the notice and registration mechanics above.



The division of labour, line by line

Carbon	Consult for Africa	The hospital
Sets the credit policy and the facility limit for each hospital	Brings the hospitals, verifies every claim before it is funded	Assigns or charges the funded claims, and tells the plans where to pay
Advances 60% of verified value within 48 hours	Reconciles and recovers with the plans	Provides records, and repairs claims we send back
Holds the collection account the plans pay into	Reports every claim's status to Carbon and the hospital	Repays any advance on a claim rejected for a reason it controlled
Prices the discount	Keeps the claims data clean enough to lend on	Submits its whole book with each plan, not only its worst claims
Registers at the NCR and holds the security	Runs the conflicts policy for any payer it advises	Keeps submitting through the desk, not around it



What the market pays for money like this

Reference point	Price	Source
Bank invoice discounting (FirstBank)	32% a year plus a 1% fee, 60-day tenor	Published product terms
Nigerian invoice finance, general	1.5% to 4% a month, plus 0.5% to 2.5% fees	Market survey, September 2026
US medical receivables factoring	70% to 90% advanced; 1% to 5% per 30 days	Industry reports
CBN monetary policy rate	23%, cut from 26.5% on 22 September 2026	CBN

ILLUSTRATION: N100M OF VERIFIED CLAIMS, 60% ADVANCED, ONE 60-DAY CYCLE

Monthly discount Carbon sets	1.5%	2.5%	3.5%
Deployed	N60.0m	N60.0m	N60.0m
Discount income per cycle	N1.8m	N3.0m	N4.2m
Over six cycles a year	N10.8m	N18.0m	N25.2m

The discount is Carbon's to price. Opening at 60% leaves a cushion of 40% of verified value against tariff cuts, late claims and the discount itself, and the rate can rise for a plan once it has paid on time for two or three cycles.



Nobody in Nigeria joins verification, recovery and funding.

Who	What they do	What it tells us
Curacel	Claims processing and fraud detection for insurers and HMOs, linking 800+ hospitals. Announced provider cash advances in 2021	The rails exist; we could not confirm the advance product is live
Medismarts	Claims platform used by 20 to 25 HMOs, about 8.4m claims a year	Sits on the HMO side, not the hospital's
Medical Credit Fund and uniBank (Ghana, 2016)	NHIS claims pre-financing: eligible claims assigned to the bank, a discounted amount paid out	The closest precedent in West Africa. MCF reports 94% to 96% repayment across its health SME book
Generic SME lenders	Unsecured working capital	Lidya's closure in 2025 is the cautionary tale

The gap is a lender that can trust what it is funding. That trust comes from verifying the claim and owning the recovery, which is the part we do.



What could go wrong, and what stops it

Risk	What it looks like	Control
Adverse selection	Hospitals send their weakest claims for funding	Whole-book submission per plan to qualify; only verified PASS claims funded
Fraud and duplicates	Padded or invented claims; the same claim funded twice	Duplicate checks on payer, amount and date; NCR search before funding; hospital warranty with recourse
Payer failure	A plan stops paying or loses accreditation	Per-payer limits and the 40% concentration cap; lower or no advance on weak payers
Set-off	A plan deducts an old overpayment from a funded claim	Vet for open clawbacks; serve notice as soon as a set-off risk appears
Plan pushback	A plan slows authorisations for hospitals that use the desk	Reconciliation first; notices only when needed; the regulator only after the relationship route
Conflict of interest	We recover from a payer CFA also advises	Written policy: that payer is never advanced against, and both sides are told in writing
Data protection	Health data leaves the hospital	No patient names held or processed; claim references only; a data protection impact assessment before the pilot



Ninety days, eight to twelve hospitals, four numbers.

8 to 12

private hospitals in Lagos

N100m to 200m

of verified claims

90 days

from first batch to review

4

numbers that decide what happens next

Measure	What it tells Carbon
First-pass acceptance	Share of verified claims the plan pays without query. The quality of our verification
Days to cash	Submission to payment into the collection account. How long Carbon's money is out
Recovery rate	Share of disputed value recovered. The quality of our recovery
Loss rate	Advances not repaid by the plan or the hospital. The number that sets the price

Weeks 0 to 2

Terms, counsel, collection accounts, data agreement

Weeks 2 to 4

Free reviews; first hospitals signed; first batches verified

Weeks 4 to 12

Funding cycles; weekly shared dashboard

Day 90

Decide rate, recourse and facility size on our own numbers

Stop-loss and step-up triggers are agreed before the first advance: the loss rate that ends the pilot, and the payment record that lifts a plan's advance rate.



Three lines that follow once the claims history exists

Surgery finance for patients

Our doctor network sees patients at the point where surgery is recommended and priced. The money goes straight to the facility, the surgeon confirms the procedure and the price is a fixed package, which is how this kind of lending keeps defaults low. Carbon's interest, and a natural second line.

Doctors' pending claims

Specialists in private practice wait on the same plans as the hospitals do. The same verification and the same advance, sized for an individual practice.

Claims submission, done right first time

Once a hospital's backlog is cleared, we take over its monthly claims preparation, so fewer claims are queried in the first place and the funded book gets cleaner every month.

The order matters. Recovery builds a claim-level payment history for every hospital and every plan, and that history is what lets Carbon price each of the next three lines with evidence rather than hope.

Four things to settle, and then a first batch

- **The route.** Carbon lends against the claims, or buys them. Counsel on both sides to confirm.
- **The pilot.** Facility size, the hospitals, and the stop-loss and step-up triggers.
- **The price.** How Carbon sets the discount, and what moves it.
- **The data.** What Carbon needs to underwrite, in what form, and how often.

Then, within a month: signed pilot terms, the first hospitals reviewed, and a first verified batch ready to fund.

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